



Barcode No	89781234	Lab No	16952502090006
Patient Name	Mrs.CHANCHAL MALHOTRA	Reg Date	09/Feb/2025 05:45PM
Age/Sex	63 YRS/Female	Sample Coll. Date	09/Feb/2025 04:51 PM
Referred By	SELF	Sample Rec.Date	10/Feb/2025 07:05 AM
		Report Date	10/Feb/2025 09:10AM

.IMMUNO BIOCHEMISTRY-1

Test Name With Methodology	Result	Unit	Biological Ref.Interval
Folic Acid [Folate]			
Folate	>20	ng/ml	2.6 - 12.2

Serum, Electro Chemi Luminescent Immuno Assay

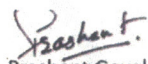
Interpretation:

1. Decreased levels of folic acid are seen in megaloblastic anemia, malnutrition, alcoholism, and inadequate dietary intake are associated with a higher risk of fetal malformations during pregnancy.
2. In patients taking methotrexate therapy, antibodies formed may interfere with the assay.

Comments:

Folate (folic acid) is vital for normal cellular functions and plays an essential role in nucleic acid synthesis, methionine regeneration, shuttling and redox reactions of one carbon units required for normal metabolism and regulation. Folate deficiency can be caused by decreased nutritional intake, poor absorption of ingested folate in the intestine or increased demand of folate, for example during physical activity or pregnancy. Deficiency of folate can also be a result of liver diseases or impaired folate metabolism due to genetic defects or drug interactions.

Hemolysis may significantly increase folate values due to high concentrations of folate in red blood cells. Therefore, hemolyzed samples are not suitable for use in this assay. Samples for folate determinations should be collected from fasting persons. Samples should not be taken from patients receiving therapy with high biotin doses (i.e. > 5 mg/day) until at least 8 hours following the last biotin administration. For diagnostic purposes, the results should always be assessed in conjunction with RBC folate, the patient's medical history, clinical examination, and other findings.


Dr. Prashant Goyal (DCP)
(Director & Chief Pathologist)
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